4.11 Pericarditis and Cardiac Tamponade*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The chest pain is sharp and worse when I lie down." **Objective** • Pericardial friction rub • Pain eases when leaning forward	**ACTUAL** Acute pain related to pericardial inflammation as evidenced by sharp positional chest pain and a friction rub.	• Pain at or below 3/10 within 24 hours.	• Sit him up, leaning forward. • Give the prescribed anti-inflammatory drug. • Assess pain at regular intervals.	• Achieved when pain is controlled.
Subjective • "I feel breathless and faint." **Objective** • Hypotension, distended neck veins, muffled heart sounds • Pulsus paradoxus	**ACTUAL** Ineffective peripheral tissue perfusion related to reduced ventricular filling from pericardial compression as evidenced by hypotension, faintness and pulsus paradoxus.	• BP restored after pericardiocentesis.	• Report signs of tamponade at once. • Prepare for urgent pericardiocentesis. • Keep vital signs and ECG on continuous monitoring.	• Outcome reached when BP is restored.
Subjective *No specific complaint* **Objective** • Pericardial effusion on echocardiography • Heart rate climbing	**RISK** Risk for decreased cardiac output as evidenced by risk factors: enlarging pericardial effusion.	• Signs of tamponade are recognized early and reported immediately.	• Check BP, HR and neck veins every 1–2 hours. • Test for pulsus paradoxus. • Keep a pericardiocentesis set ready. • Report a falling BP, rising HR or muffled heart sounds without delay.	• Considered met if tamponade does not develop.

*References for this care plan: [17], [32], [35], [128]. Full entries are listed in the References section of the CPMN book.